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**SCHOOL OF PUBLIC HEALTH, COLLEGE OF HEALTH SCIENCES,
ADDIS ABABA UNIVERSITY**

**Final Reproductive Health Examination Questions for Year IV
Medical Students**

June 22, 2013

INSTRUCTION:

THIS IS A FINAL EXAMINATION QUESTIONS ON **REPRODUCTIVE HEALTH**
COURSE. ATTEMPT ALL QUESTIONS. DON'T FORGET TO WRITE YOUR NAME AND
I.D. NUMBER ON EACH OF THE PAGES.

TIME ALLOWED: 1:30 HOURS

Final RH exam

...containing the correct answer.

Part 1: Concept of Reproductive Health and Family Planning

1. One of the following has no relationship with the concept of Family planning?
a. Primary Health Care - *me H*
b. Safe motherhood ✓
c. UN declaration on human rights day, December 1967
d. Millennium Development Goals ✓
e. None of the above
2. Which of the following approach of Family planning is related to reduction of the consequences of high fertility on mothers and children? — *Health rationale*
a. A concept that was emphasized during the 1960s and 1970s
b. Demographic rationale of Family planning
c. The focus of Family planning during the era of health rationale of Family planning — *1980*
d. Main focus of the 1994 ICPD in Cairo
e. None of the above
3. The strategic approach in the successful and sustainable implementation of prevention of unwanted pregnancy would be:
a. Individualization of contraceptive choices ✓
b. Focusing on best contraceptive methods
c. Improving the capacity of health workers in rendering Family planning service ✓
d. Improving the logistic aspect of contraceptive commodities
e. None of the above *all*
4. According to the 2011 Ethiopian Demographic Health Survey, the Contraceptive prevalence rate (CPR) has almost been doubled from what it was in 2005 EDHS.
a. True
b. False
5. Nowadays, one of the following Family planning service delivery has been an obsolete strategy in the Country
a. Workplace provision of Family planning methods
b. Social marketing (commercial retail sales) on a subsidized pricing
c. Facility based service through an exemption approach

- d. Distribution of contraceptives through a community based strategy
None of the above
6. The total fertility rate (TFR) is expressed as
a. Number of children born to 1000 women
b. Number of children per woman
c. Number of all births in a specified community per 1000 population
d. The number of children born to women in a specific age groups
e. None of the above
7. All are elements of good Family planning counselling except -
a. Considering "one size may not fit all" approach to clients
b. Providing group approach on clients with similar characteristics
c. Considering tailored approach to the individual client
d. Giving comprehensive FP counseling though methods are not available
e. None of the above
8. Which of the following barrier to Family planning use relatively require an intensive resource and would continue to be a challenge in the long run?
a. Limited access to and availability of FP products and services
b. Limited alternative delivery approach
c. Lack of skilled providers both in number and quality
d. Cultural and religious barriers including gender issues
e. None of the above
9. Proper implementation of Family planning program would better contribute to the targets set for MDG 4 and MDG 5.
A. Correct B. Not correct
10. Which of the following pairs are the unmet needs for family planning for 2005 and 2011 respectively?
a. 34% and 29%
b. 34% and 14%
c. 29% and 14%
d. 34% and 25%
e. None of the above

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Part II: Maternal Health

11. Which one of the following is not true about "Maternal Mortality"?
- a. The death of a woman while pregnant or within 42 days after termination of pregnancy ✓
 - b. Does not depend on site and duration of pregnancies ✓
 - c. Death of mothers which is not caused by accidental or incidental causes ✓
 - d. Caused or aggravated by pregnancy ✓
 - ☒ e. Death of a woman from direct or indirect obstetric causes more than 42 days but less than one year after the termination of pregnancy ✓
12. Which one of the following is not an Intermediate / contributing Factor for maternal mortality?
- a. Poor access of health services ✓
 - ☒ b. Poverty
 - c. Poor quality of health services ✓
 - d. Age at delivery ✓
 - e. Gravidity/parity of woman heart starts
13. Which one of the following is incorrect about direct cause of maternal mortality?
- ☒ a. Those resulting from previous existing diseases or diseases that develop during pregnancy ✓
 - b. Results from obstetric complications and interventions
 - c. A chain of events resulting from omissions or incorrect treatment
 - d. Accounts for high proportion of maternal death (82%)
 - e. None of the above
14. One of the following obstetric complication has too short median time period between the onset of complication to death of a mother
- a. Puerperal sepsis - 6 days
 - b. Eclampsia - 2 days
 - ☒ c. Post Partum Hemorrhage - 2 hrs
 - d. Ruptured uterus - 1 day
 - e. Obstructed labor - 2 days
15. Which one of the following causes of maternal mortality accounts for high proportion in Ethiopia?
- ☒ a. Ectopic pregnancy
 - b. Complication of abortion - 6%
 - c. Antepartum hemorrhage - 11%
 - ☒ d. Obstructed labour - 19%
 - e. Post partum sepsis 12% 16%
16. All are causes for delay in decision to seek care for an obstetric complication except
- a. Inadequate knowledge about danger sign during pregnancy and labor ✓
 - ☒ b. Quality of care
 - c. Lack of money ✓
 - d. Cultural /traditional practices ✓
 - e. Lack of information ✓
17. Which one of the following does not indicate maternal mortality ratio related with MDGs?

- a. An average annual decline was 3.1% *5.5% same*
- b. Maternal Mortality Ratio was 240 per 100000 live births in 2010
- ☒ c. Maternal deaths worldwide have dropped by 57% Since 1990 *45%*
- d. The target is to reduce maternal mortality ratio by 75% until 2015
- e. None of the above
18. Which one of the following maternal mortality ratio indicators is incorrectly matched with the corresponding statistics in Ethiopia based on EDHS?
- ☒ a. 873 maternal deaths/100,000LBs in 2005 *673*
- b. 676 maternal deaths/100,000LBs in 2011
- c. 871 maternal deaths/100,000LBs in 2000 *45%*
- d. All of the above
- e. None of the above
19. Why is Safe Motherhood initiative important?
- a. Magnitude of the problem ✓
- b. Greatest disparity between developed and less developed countries among common basic health status indicators ✓
- c. Lose of mother's contribution to household management and provision of care for children when she is sick or dies
- d. Maternal death is generally avoidable
- ☒ e. All of the above
20. All are true about lessons learned from Safe Motherhood Initiative of 10 yrs evaluation except
- a. Major obstetric complications are not predictable ✓
- b. Risk assessment has not worked ✓
- c. Some obstetric complications are not preventable ✓
- d. Early detection of complication is possible but needs skilled attendant ✓
- ☒ e. None of the above
21. Which one of the following is not aim of ANC?
- a. Promoting and maintaining the physical, mental & social health of mother & baby ✓
- b. Detecting and treating complications during pregnancy ✓
- c. Helps to develop birth preparedness and complication readiness plan ✓
- d. All of the above
- ☒ e. None of the above
22. All of the following statements are true about risk approach in ANC except
- a. Identify pregnant women at risk as early as possible and intervene in order to reduce the risk
- b. Optimal use of existing resources for the benefit of the majority ✓
- ☒ c. Considers all pregnancies as "at risk" of developing a complication *Local approach*
- d. Minimum of care for all and intensive care for high risk ✓
- e. Diversion of limited resources to those who most need them ✓
23. Which one of the following is not correctly matched?

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- a. Third visit : 20-32 weeks ✓ - 216
b. Progress monitoring : Process indicator ✓ 24-28
c. All pregnant women at first visit : Basic component of ANC 26-32
d. Current Strategic focus: Skilled attendant at birth ✓ 36-38
e. Partograph: Prolonged labour ✓ all - 7
24. One of the following is not included in Basic Emergency Obstetric Care
- a. Parenteral oxytocic drugs administration
 - ☒ b. Blood transfusions ✓ Co EME
 - c. Manual removal of placenta
 - d. Removal of retained products
 - e. Administer parenteral anticonvulsants for pre-eclampsia and eclampsia
25. Which one of the following is not the component of postnatal care service provided for newborn?
- a. Early detection and management of Complications
 - b. Support and establish breastfeeding
 - ☒ c. Information and services for family planning, nutrition and hygiene
 - d. Immunization
 - e. Growth monitoring

Adolescent Health

26. One of the following is not among the macro-level protection risk factors affecting adolescent and youth health and development in general and their Reproductive Health in particular.
- a. Adult employment rates in the country
 - b. Existing political situation in a country
 - c. Absence or presence of youth health policies in a country
 - d. Gross domestic Product of the nation
 - e. None of the above
27. One of the following is not true about adolescent and youth health programs.
- a. It requires a meaningful involvement of youth in program's design and implementation
 - b. Creation of safer and supportive environment for adolescent and youth is among the interventions required by the program
 - c. Residential centres cannot serve as an intervention setting as most residential areas may stigmatize adolescents and youths
 - d. All of the above
 - e. None of the above
28. Taking EDHS 2000, 2005 and 2011 as reference points, which of the following statement is (are) true about adolescent and youth reproductive health?
- a. The proportion of youth who began childbearing at the age of 15 years increases over years
 - b. Most adolescent girls die from pregnancy-related causes than from any other causes
 - c. Young men tend to marry earlier in Ethiopia compared to their female counterparts
 - d. All of the above
 - e. None of the above
29. All of the following are true about adolescence except.
- a. It is characterized by rapid development and changes of physical and mental development
 - b. It is characterized by acquiring new capacities new body image and a new sense of self (learning, thinking, intellectuality)
 - c. More health and development challenges than opportunities are observed during the period of adolescence
 - d. All of the above
 - e. None of the above
30. One of the following is correct.
- a. Puberty marks the beginning of Adolescence
 - b. Age at first marriage and/or childbearing age can mark the beginning of adolescence
 - c. The end point to the transition societal specific
 - d. A and C
 - e. All of the above

31. Governments and Non-governmental Organizations (NGOs) including other stakeholders are currently emphasizing the need to focus on adolescents and adolescent health and other related issues. The main reasons are



- a. Adolescents have different needs from adults and need different intervention strategies ✓
- b. Adolescents make 25% of the population in the world and these are the future adults who need care and special attention
- c. Adolescents are exposed to different risky and unfriendly environments during their stage of development and hence they need special attention from all stakeholders ✓
- d. Adolescent health programs are rich and funds can easily be obtained for those stakeholders focusing on adolescents
- e. All of the above

32. Which of the following statement(s) is (are) true about adolescence and adolescent health problems?

- a. The major causes of adolescent sexual and reproductive health problems are *early and unprotected sexual intercourse* which end up in *early and usually unwanted pregnancies* ✓
- b. Lack of knowledge on physiology of the reproductive system and human sexuality is one of the main reasons for adolescent sexual and reproductive health problems ✓
- c. Unavailability and inaccessibility of youth-friendly health services and lack of knowledge on contraceptive use and methods contribute less to the occurrence of adolescent sexual and reproductive health problems ✗
- d. A and b are answers
- e. All of the above

33. One of the following is not appropriate contraceptive method for the teenagers

- a. Pills
- b. Condom
- c. Voluntary Surgical contraception •
- d. Norplant
- e. C & D are correct
- f. None

34. Which of the following is true statement

- a. Developing skill based training and business opportunities in rural areas to reduce the flight of young people to the cities could help in addressing the economic problems of adolescents. ✓
- b. High rate of reported mortality and morbidity and the less obviousness of adolescent health problems have made it to be neglected.
- c. Poverty robs adolescents' opportunities for education to acquire intellectual and vocational skills required for gainful employment. ✓
- d. A and B
- e. A and C

35. Adolescents are the healthiest group of the population

☒ True ✓

☒ False ✗

CHILD (PUBLIC) HEALTH PORTION to the FINAL WRITTEN EXAM
YEAR IV UNDERGRADUATE MD STUDENTS
SCHOOL OF MEDICINE - SCHOOL OF PUBLIC HEALTH
COLLEGE OF HEALTH SCIENCES, ADDIS ABABA UNIVERSITY
20th JUNE 2013

FULL NAME: _____

ID No.: _____

Please CHECK to ascertain COMPLETENESS (two-pager with NINETEEN QUES).

Please write your FULL NAME and ID No. on EACH PAGE.

READ CAREFULLY AND MAKE A CIRCLE ON THE MOST APPROPRIATE ANSWER

1. The Child Public Health and Pediatrics streams should not have exhibited any tangible difference(s).
a) True b) Not sure ☒ c) False
2. Essentially, the Child and Child's health as a whole are the products of the biological predetermination more than the social influence.
a) True b) Not sure ☒ c) False
3. Neonatal Health live with the overall health of the under-five children has been showing a steady improvement in the world over the last 15 years.
a) True b) Not sure ☒ c) False
4. Neonatal mortality monitoring comprises one of the core indicators of the MDG4.
☒ a) True b) Not sure c) False
5. Neonatal Mortality was estimated (reported) to have been occupying approximately 30.9% share of the overall under-five mortality burden across the globe in recent years.
a) True b) Not sure ☒ c) False
6. As much we rigorously endeavor to advance the health of our children, it simply is impractical and unfeasible to be able to simultaneously pursuing the Early Childhood Development path in Ethiopia.
a) True b) Not sure ☒ c) False
7. Late Childhood Health and Adolescent Health should, essentially, mean the same.
a) True b) Not sure ☒ c) False
8. According to the CRC any given Nation or State has an obligation to protect the health of the Child.
☒ a) True b) Not sure c) False
9. Both the Pathway to Child Survival and IMCI comprise integrative intervention strategies...
☒ a) True b) Not sure c) False

10. Meaningful enough Neonatal Mortality Rate reduction efforts will, equally, hinge upon the society's capacity of ensuring Skilled Birth Attendance (SBA) system in particular.
☒ a) True ☐ b) Not sure ☐ c) False
11. Thanks to the impressive progresses made across the last several decades, mortality reduction, specifically, among the Under-Five years of age children has become of less concern in Ethiopia.
☐ a) True ☐ b) Not sure ☒ c) False.
12. Whether according to the Ethiopian Demographic and Health Survey or other pertinent references "Child Mortality" and "Under-Five Years Children Mortality" rates bear similar interpretations.
☐ a) True ☐ b) Not sure ☒ c) False
13. The envisaged success of the National Child Survival Strategy for Ethiopia, among others, heavily counts upon the Health Services Extension Programme.
☒ a) True ☐ b) Not sure ☐ c) False
14. Integration of the RMNCH Programmes and Services Continuum was often proven impractical for health system settings typical of the contemporary Ethiopia.
☒ a) True ☐ b) Not sure ☐ c) False,
15. Based on the EDHS, the reduction of neonatal, infant, and under-five children's mortalities in Ethiopia exhibited typical comparability and uniformity for trends across the indicators 2000 through 2011.
☐ a) True ☐ b) Not sure ☒ c) False,
16. It could have been impossible to expect meaningful reduction of mortality among the under-five years' children, particularly, in the face suboptimal immunization coverage as has been the case in Ethiopia.
☒ a) True ☐ b) Not sure ☒ c) False
17. Equitable child health programming and services access and coverage are no more concerns for Ethiopia.
☐ a) True ☐ b) Not sure ☒ c) False
18. Child health interventions/services make up the core of the Rural Community Health Training Programme of the SCIM-SPH joint attachment (e.g. the classical Ziway RCHTP).
☒ a) True ☐ b) Not sure ☐ c) False
19. If you were to define/describe the Child Public Health notion very concisely, then:
is science and art of public health branch specifically
deals with, comprehensive and holistic health care and
of child health population in a given community or
society

Please DO NOT FORGET TO PUT YOUR SIGNATURE AGAINST YOUR NAME AND ID!

WISHING EVERY POSSIBLE BEST TO YOUR WHOLE FUTURE!!